Secondary Victims

Psychiatric injury in negligence in England and Wales. Alcock, Taylor v A Novo and the Supreme Court's decision in Paul [2024] UKSC 1.

1. A claim for the claimant's own illness

A **secondary victim** suffers injury through witnessing an accident, or its immediate aftermath, in which someone closely connected to them is killed, injured or endangered. The claim is for the claimant's own medically recognised psychiatric illness, not compensation for the other person's injuries. Ordinary grief or distress alone is insufficient. [1]

Unlike a primary victim in the ordinary accident category, the secondary victim is not relying on personal exposure to foreseeable physical injury. The fact that a loved one was injured by negligence does not automatically establish a duty to protect everyone affected by the news.

2. A diagnosis and negligence are not the whole test

A claimant must establish the relevant duty of care, breach, recognised injury and causation. Psychiatric illness must also have been reasonably foreseeable in a person of ordinary fortitude in the claimant's position. Do not transfer Page v Smith's primary-victim foreseeability rule automatically to secondary victims.

The **Alcock control mechanisms** additionally examine the relationship, closeness to the accident, and how it was perceived. They limit the scope of the duty. They are not merely a way to decide whether the claimant's feelings are understandable. [1, 2]

> **A practical framework:** Identify an accident within the established category → establish foreseeable, recognised psychiatric illness caused by witnessing it or its immediate aftermath → examine close ties, proximity and direct perception. The underlying negligence and a duty to this claimant still matter.

3. Three separate connections

Personal: What relationship connects the claimant to the person harmed or endangered? **Temporal and spatial:** Was the claimant present at the accident or its immediate aftermath? **Perceptual:** Did the claimant directly experience that accident or aftermath, rather than simply receive someone else's account?

Duty of care: a legal obligation to take reasonable care towards the claimant. **Proximity:** the legally relevant closeness of the relationship or events. **Immediate aftermath:** the accident's directly connected aftermath, not every later consequence.

Alcock and the three controls

4. Alcock v Chief Constable of South Yorkshire Police

Case: [1992] 1 AC 310; judgment delivered in 1991. Relatives of people killed or endangered in the Hillsborough disaster brought psychiatric-injury claims. Some were at the stadium; others saw television coverage or later identified bodies. The police admitted negligence towards those physically harmed, but the claimants had to establish duties owed to themselves.

Outcome: All the claims before the House of Lords failed. Different claimants failed different requirements. Being a relative, being somewhere in the stadium or suffering a genuine illness did not by itself satisfy the complete test. [1, 2]

5. Close ties of love and affection

The claimant must have a close tie of love and affection with the person killed, injured or endangered. Such a tie is generally presumed between spouses and between parents and children. Other relationships, including siblings, require evidence; they are not automatically excluded, but a family label alone is not proof.

A claim involving a close friend would depend on proof of a sufficiently close tie and every other requirement. “We were best friends” is not an automatic entitlement.

6. Closeness in time and space

The claimant must be close to the **accident or its immediate aftermath**. McLoughlin v O’Brian [1983] 1 AC 410 shows that an immediate encounter with an injured family at hospital can qualify. In Alcock, later mortuary identification did not qualify on the facts. There is no universal fixed number of hours. [1, 2]

7. Direct perception

The illness must result from direct perception of the accident or aftermath, ordinarily through sight or hearing, rather than a report from another person. The television broadcasts in Alcock did not satisfy this requirement. This should not be rewritten as a comprehensive ruling on every conceivable form of live communication.

> **The controls work together:** A parent may establish the relationship but fail proximity. A sibling at the scene may fail to prove the required tie. Presence and relationship do not dispense with recognised illness, foreseeability or causation.

Taylor v A Novo

8. Taylor v A Novo (UK) Ltd [2013] EWCA Civ 194

Facts: Mrs Taylor was injured at work when racking boards fell on her. Her employer admitted negligence. She appeared to be recovering, but three weeks later she collapsed and died at home from a pulmonary embolism caused by deep-vein thrombosis resulting from the accident injuries.

Her daughter witnessed the death and developed post-traumatic stress disorder (PTSD). She had not witnessed the workplace accident or its immediate aftermath. She had a close relationship with her mother and directly saw the later death, but those facts did not resolve the proximity requirement.

Outcome: The employer's appeal succeeded in the Court of Appeal and the daughter's claim failed. The relevant event was the workplace accident, not a separate death three weeks later. The later collapse did not become a new qualifying accident merely because it caused the daughter's illness. [3]

> **The sequence:** Accident at work → apparent recovery → collapse and death three weeks later, witnessed by the daughter. The daughter was present for the last event, but not for the accident or its immediate aftermath.

9. Correctly identify the event requiring proximity

Do not shorten the rule to "the claimant must witness the negligent act". The defendant's careless conduct and the accident it causes can happen at different times. The relevant proximity is to the accident or its immediate aftermath, not necessarily to the earlier breach. The Supreme Court expressly clarified this in Paul. [1]

Invented contrast: A contractor negligently fixes masonry months before it falls and injures a child. A parent sees the fall. The parent did not need to watch the contractor make the original mistake. The remaining secondary-victim requirements must still be proved.

10. What Taylor does not decide

The daughter's failure does not mean that her illness was unreal, or that three weeks is a universal time limit. It shows why the accident must be distinguished from a later consequence. Causation of the mother's death and a duty to protect the daughter from her own illness are separate legal questions.

Paul: the decision and its meaning

11. Paul v Royal Wolverhampton NHS Trust

Decision: [2024] UKSC 1, delivered on 11 January 2024. It is no longer a pending appeal. Paul was heard with Polmear and Purchase. The claims concerned psychiatric illness after witnessing a relative's death or a medical crisis resulting from alleged earlier failures to diagnose and treat disease. [1]

Paul's facts: Two daughters saw their father collapse in the street and the attempts to resuscitate him. He died. They alleged that negligent failure to diagnose and treat coronary disease during a hospital admission about fourteen months earlier had caused his death. For deciding whether the claims could proceed, relevant pleaded facts were assumed, not all finally proved at a trial.

Earlier appeal: In 2022 the Court of Appeal held itself bound by Taylor v A Novo, although some judges expressed reservations. The Supreme Court then examined whether the accident-based exception should extend to these medical cases.

12. Outcome and the majority's reason

The Supreme Court dismissed all three appeals by six to one. The defendants succeeded against these secondary-victim claims. Witnessing a relative's medical crisis or death from disease was not an accident of the relevant kind. The majority declined to extend the established exception to these circumstances.

The duty to provide proper medical care to a patient did not, in these circumstances, extend to protecting family members from psychiatric injury caused by witnessing the manifestation of the patient's disease or death. This does not erase the patient's own claim or create blanket immunity for every accident in healthcare.

> **The crucial point is not simply the fourteen-month gap.** An accident may follow negligence much later. Paul's key distinction is between witnessing a qualifying accident and witnessing a medical crisis from disease. Taylor's result was upheld; it was not overruled.

13. Two outdated additions to avoid

Paul also rejected separate requirements to prove a "sudden shock to the nervous system" or an event sufficiently "horrifying" by some additional standard. The claimant still needs a causal connection between witnessing the accident or aftermath and recognised illness. Removing those extra requirements does not remove the accident-based limit or the Alcock controls. [1, paras 71-78, 95]

Check your understanding

1. Is the claim for the injured relative's loss?
Answer: No. The secondary victim seeks compensation for their own psychiatric illness.

2. Does a brother or sister qualify automatically?
Answer: No. A sufficiently close tie must be proved, together with every other requirement. Siblings are not automatically excluded either.

3. Why did the daughter lose in Taylor v A Novo?
Answer: She witnessed a later death, not the workplace accident or its immediate aftermath. The later death did not itself become the qualifying accident.

4. Must a parent witness a builder's earlier negligent work?
Answer: No. The required proximity is to the accident or its immediate aftermath, not necessarily to the earlier negligence.

5. Did the Supreme Court allow the families' claims in Paul?
Answer: No. It dismissed all three appeals in 2024 and declined to extend the accident-based exception to the medical crises in question.

6. Are "sudden shock" and a separately "horrifying event" additional current tests?
Answer: No. Paul rejected those extra requirements. Recognised illness, causation, foreseeability and the applicable limits remain.

7. Invented example: a mother sees a negligently driven car seriously injure her child and develops PTSD. Is success automatic?
Answer: No. This fits the ordinary accident framework, but the close tie, direct perception, proximity, foreseeable illness, diagnosis and causation must still be established.

Sources and further reading

[1] *Paul v Royal Wolverhampton NHS Trust; Polmear v Royal Cornwall Hospitals NHS Trust; Purchase v Ahmed* [2024] UKSC 1: paras 26-51 (earlier rules), 71-78 (shock and horror), 83-96 (Taylor and timing), 138-143 (medical duty and result). Supreme Court judgment.

[2] *Alcock v Chief Constable of South Yorkshire Police* [1991] UKHL 5; [1992] 1 AC 310. The controls are summarised in *White* [1999] 2 AC 455 and in *Paul*, para 45. *McLoughlin v O'Brian* [1982] UKHL 3; [1983] 1 AC 410 concerns the immediate aftermath.

[3] *Taylor v A Novo (UK) Ltd* [2013] EWCA Civ 194; [2014] QB 150. Court of Appeal judgment, The National Archives. Report years can differ from judgment years.